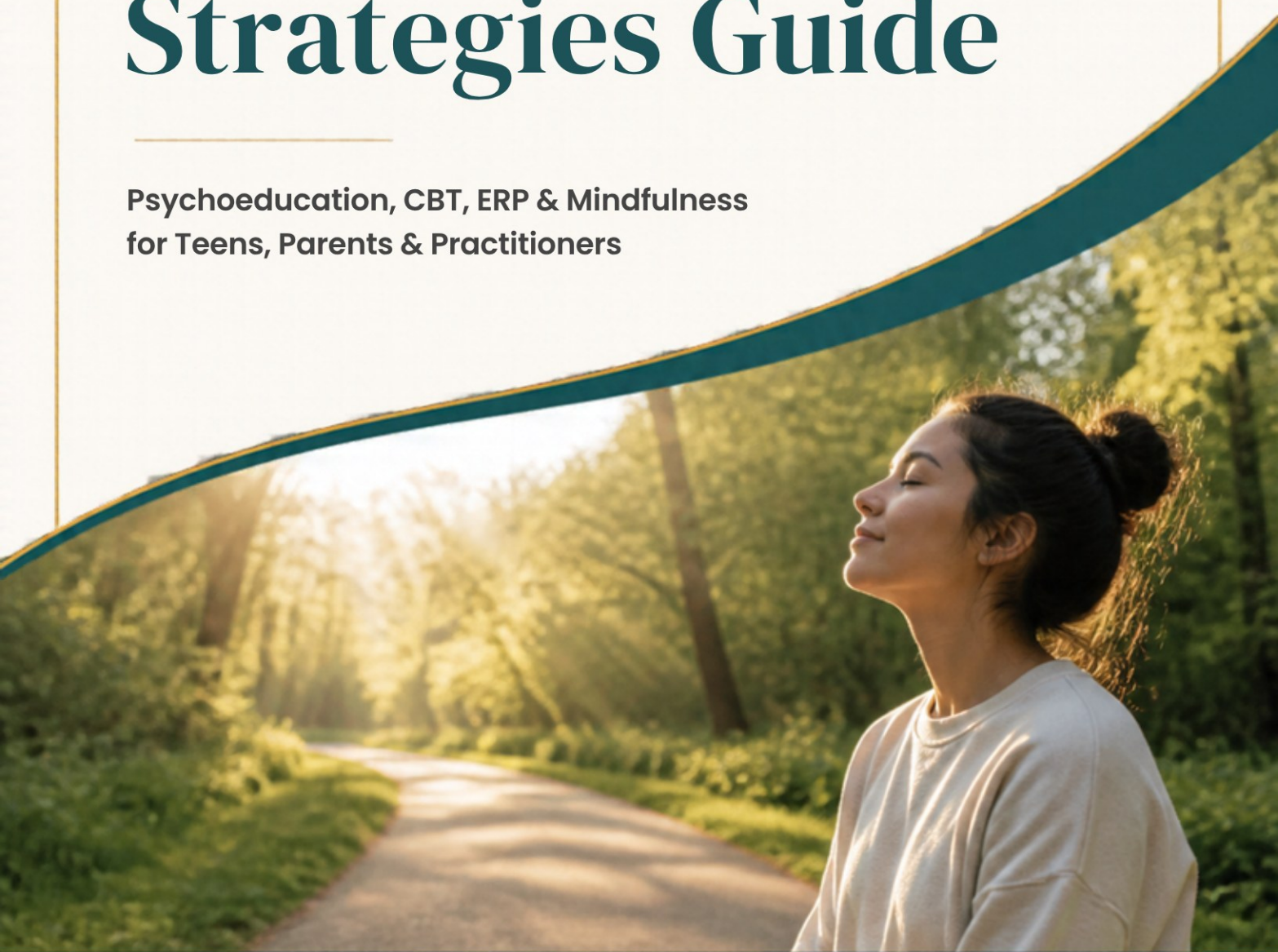
ANCHORED INSIDE
COUNSELING SERVICES

Understanding OCD: Skills & Strategies Guide

Psychoeducation, CBT, ERP & Mindfulness
for Teens, Parents & Practitioners



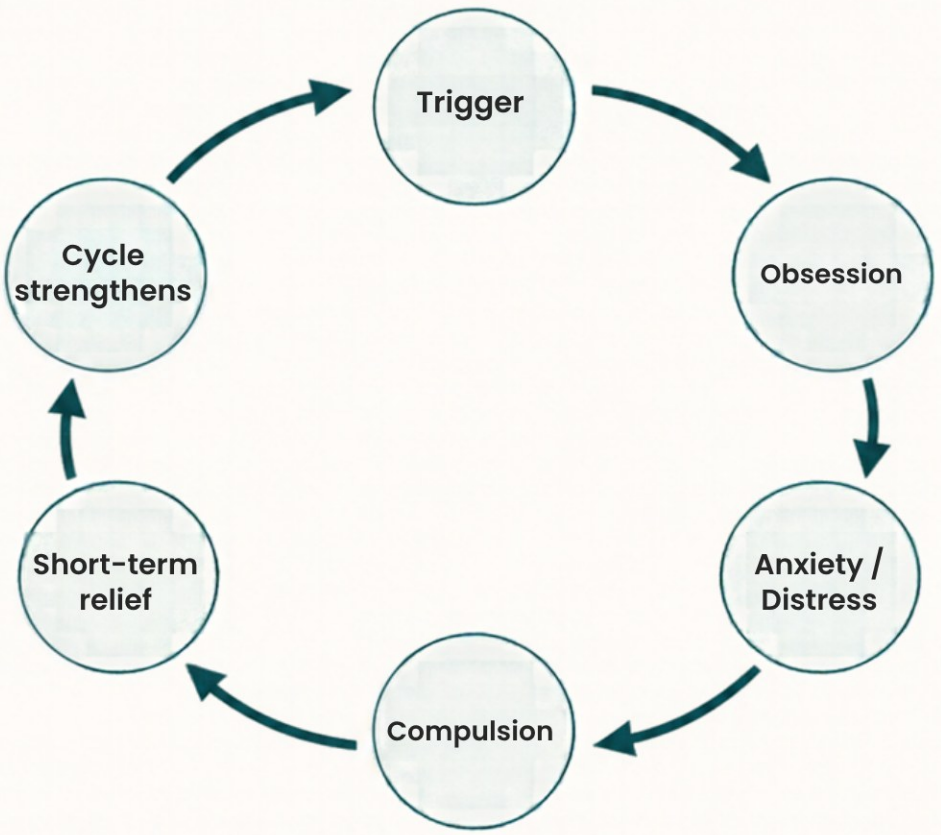
What Is OCD?

Obsessions are unwanted, intrusive thoughts, images or urges that cause distress.

Compulsions are actions or mental rituals done to reduce that distress or prevent a feared outcome.

OCD affects about 1 to 2 percent of people and is treatable.

The OCD Cycle



Common Types & Myths

Common types

- Contamination and washing
- Checking and doubt
- Symmetry, order, and 'just right' feelings
- Unwanted harm, sexual, or religious thoughts (scrupulosity)
- Relationship and health worries
- Mental rituals and reassurance seeking



Myths vs. facts

MYTH	FACT
OCD is just being neat	OCD is distressing and time-consuming.
Having a scary thought means you want it	Intrusive thoughts are common and often go against a person's values.
You can just stop	OCD is a real condition that responds to treatment.

How Treatment Works: CBT & ERP

Cognitive Behavioral Therapy with Exposure and Response Prevention (ERP) is the leading evidence-based treatment; medication (SSRIs) can also help.

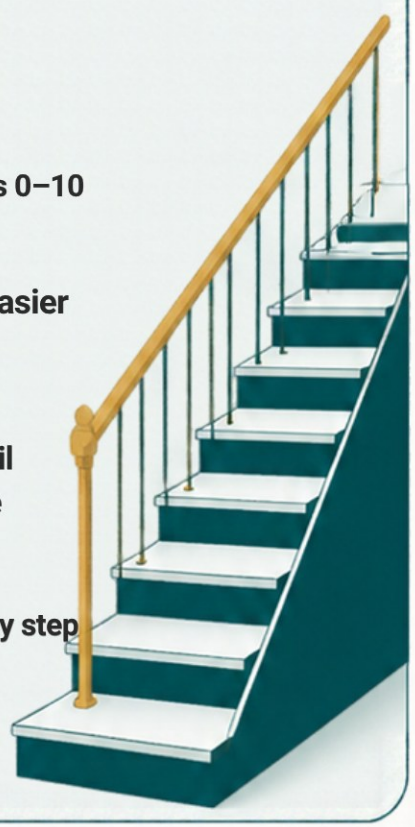
Exposure: gradually facing feared thoughts and situations.

Response prevention: choosing not to do the compulsion.

Anxiety rises, peaks, then naturally falls; the brain learns it can handle uncertainty.

The Exposure Ladder

1. List fears
2. Rate distress 0–10
3. Start with easier steps (3–4)
4. Practice until manageable
5. Climb step by step



Recovery happens through steady, courageous practice. You don't have to do it alone—we'll take each step together.

The Phases of Therapy

OCD treatment works best as a step-by-step climb. Each phase builds the skills you need for the next one, so that by the time you face your fears with Exposure and Response Prevention (ERP), you have a full toolbox to lean on. Moving through the phases is not a race. Some people move quickly, others take longer, and it is normal to circle back.

PHASE 1

Grounding Techniques

Learn to calm the body and come back to the present moment when anxiety spikes. *Goal: "I can steady myself."*

PHASE 2

Emotion Regulation Techniques

Name, understand, and ride out strong feelings without needing a compulsion to make them stop. *Goal: "I can feel this and still be okay."*

PHASE 3

Cognitive Coping Techniques

Spot OCD's thinking traps, step back from intrusive thoughts, and practice living with uncertainty. *Goal: "A thought is not a fact."*

PHASE 4

Exposure Hierarchy & ERP

Build a fear ladder, then climb it one rung at a time, facing triggers while resisting rituals. ERP is the gold-standard treatment for OCD. *Goal: "I can face it and let the anxiety pass on its own."*

A note on the early phases

Grounding, emotion regulation, and cognitive skills are **supports**, not the cure. They help you tolerate ERP. Be careful they do not turn into new rituals: they are used to stay present with discomfort, not to make it disappear or to "neutralize" a thought. Your therapist may also weave the phases together rather than doing them strictly in order.

Phase 1: Grounding

Goal

Lower the body's alarm so you can think clearly and stay in the present moment.

Why it comes first

When the nervous system is flooded, learning is hard. Grounding gives you a quick, reliable way to steady yourself before tackling harder work.

Skills to practice

1. **5-4-3-2-1 senses.** Name 5 things you see, 4 you can touch, 3 you hear, 2 you smell, and 1 you taste. Say them slowly, out loud or in your head.
2. **Box breathing.** Breathe in for 4, hold for 4, out for 4, hold for 4. Repeat 4 rounds. Keep the exhale soft and slow.
3. **Feet on the floor.** Press both feet down, notice the floor holding you up, and feel the chair at your back. Wiggle your toes and name where you are and today's date.
4. **Temperature reset.** Hold something cold (ice, a cold drink) or splash cool water on your face for 15–30 seconds to slow the heart rate.
5. **Anchor object.** Carry a small stone or bracelet. When anxious, notice its weight, texture, and temperature for one minute.
6. **Progressive muscle relaxation.** Tense one muscle group for 5 seconds, then release for 10. Move from feet to face.

Practice plan

- Practice one skill twice a day when calm (it works better in a storm if you trained in sunshine).
- Rate anxiety 0–10 before and after.
- Families: practice together at bedtime or in the car.

Ready for Phase 2 when...

- You can name 2–3 skills that help.
- You can use one on your own during a moderate spike.
- Anxiety comes down at least a little after practice.

Phase 2: Emotion Regulation

Goal
Understand your feelings and learn that you can ride out anxiety, guilt, or disgust without doing a compulsion.

Why it comes now
Once you can steady your body, the next step is handling the emotions OCD stirs up. ERP asks you to sit with discomfort, so this phase builds that muscle.

Skills to practice

1. **Name it to tame it.** Label the feeling and its size: "I'm feeling anxiety at a 7." Naming a feeling helps calm the brain's alarm center.
2. **Urge surfing.** Picture the urge to do a ritual as a wave. Notice it rise, peak, and fall. Breathe and watch, without acting, for 2–5 minutes.
3. **Feelings thermometer.** Use a 0–10 scale several times a day. Notice what pushes the number up and what helps it come down on its own.
4. **TIPP for big waves. T**emperature (cold water), **I**ntense exercise (1–2 minutes of jumping jacks), **P**aced breathing (longer exhale), **P**aired muscle relaxation.
5. **Self-compassion statement.** "This is hard. Lots of people feel this. I can be kind to myself while I feel it."
6. **Build up the basics.** Regular sleep, meals, movement, and fun activities each day make emotions less intense and easier to manage.

Practice plan

- Keep a daily feelings log: trigger, feeling, rating, what helped.
- Practice urge surfing once a day with a small urge.
- Parents: reflect feelings ("You look really worried") rather than fixing or reassuring.

Ready for Phase 3 when...

- You can name feelings and rate their size.
- You have ridden out at least a few small urges.
- You notice feelings rise and fall on their own.

Phase 3: Cognitive Coping

Goal

Change your relationship with intrusive thoughts: notice them, label them as OCD, and let them be there without arguing or obeying.

Why it comes now

With body and feelings steadier, you can step back and look at OCD's tricks. Understanding the traps makes it easier to choose ERP over rituals.

Skills to practice

1. **Externalize OCD.** Give OCD a name ("the Bully," "Worry Monster"). Say, "That's OCD talking, not me."
2. **Catch the thinking trap.** Ask: Am I overestimating danger? Taking too much responsibility? Believing a thought makes it happen? Needing 100% certainty?
3. **Cognitive defusion.** Add "I'm having the thought that..." in front of the thought, or say it in a silly voice. Thoughts become words, not orders.
4. **Probability vs. possibility.** Ask, "Is this probable, or just possible?" Then practice: "It's possible, and I can live with maybe."
5. **Uncertainty statements.** Instead of seeking reassurance, practice: "Maybe, maybe not. I'm choosing to move on anyway."
6. **Spot hidden rituals.** List mental checks, reviewing, and reassurance-seeking. Arguing with OCD forever is also a ritual; the goal is to stop debating.

Practice plan

- Thought log: intrusive thought, trap, defusion response.
- Pick one reassurance question and practice not asking it.
- Families: agree on a script: "That sounds like OCD. What would your brave self do?"

Ready for Phase 4 when...

- You can label OCD thoughts in the moment.
- You can list your main rituals, including mental ones.
- You feel willing (not fearless) to start facing triggers.

Phase 4: Exposure Hierarchy & ERP

Goal

Face feared thoughts, images, and situations on purpose, without rituals, so the brain learns it can handle anxiety and uncertainty.

Why it comes last

ERP is the gold standard for OCD and the heart of treatment. The earlier skills make it possible to stay with the discomfort long enough to learn.

Step A: Build the hierarchy

1. List triggers you avoid or ritualize around.
2. Rate each with SUDS (0 = calm, 100 = most distress ever).
3. Order them low to high; aim for 10–15 steps with small gaps.

Step B: Do ERP

4. Start near SUDS 30–40. Rate SUDS before you begin.
5. Stay in the exposure and **do not ritualize**. Notice the feeling without pushing it away.
6. Stay until the urge is weaker or you have learned something new (often 20–45 min). Rate SUDS at the end.
7. Repeat daily until it feels manageable, then climb a rung.

Response-prevention tips

- Delay rituals: start with 5 minutes, then grow it.
- Shrink them: wash for 20 seconds, not 5 minutes.
- No reassurance: families respond with support, not answers.
- Watch for mental rituals and sneaky avoidance.
- Do not use grounding to escape the exposure.

Handling setbacks

- Slips are normal and part of learning.
- Drop back a rung, then climb again.
- If you did a ritual, re-expose (“undo the undo”).
- Stress, illness, and transitions can raise symptoms for a while.
- Celebrate effort, not perfection.

Signs of progress

- Peak SUDS lower, or you tolerate it better.
- Fewer rituals, less avoidance, more time for life.
- You can design your own exposures (relapse prevention).

My Exposure Hierarchy Ladder

Write your steps from easiest (bottom) to hardest (top). Rate each with SUDS 0–100. Name the ritual you will resist.

Example ladder (contamination fears)

Step	Exposure	SUDS	Ritual to resist
6	Touch a public toilet seat, then eat a snack	95	Washing, sanitizer
5	Touch the bathroom floor, then my face	80	Washing for 1 hour
4	Touch a trash can lid, then my phone	65	Wiping my phone
3	Use a public door handle without a sleeve	50	Sanitizer right after
2	Sit on a couch in clothes worn outside	40	Changing clothes
1	Touch a doorknob at home, wait 10 min	30	Asking "Is it clean?"

My ladder

Step	Exposure (what I will face)	SUDS	Ritual to resist
10			
9			
8			
7			
6			
5			
4			
3			
2			
1			

Practice log: Date _____ Step # ___ SUDS start ___ peak ___ end ___ Minutes ___ What I learned _____

CBT Thinking Skills

Thinking Traps in OCD

- **Overestimating threat —** *"Something terrible will surely happen."*
- **Inflated responsibility —** *"If anything goes wrong it's all on me."*
- **Thought-action fusion —** *"Thinking it makes it more likely or means I'm bad."*
- **Need for certainty —** *"I must know for 100% sure."*
- **Perfectionism**



Reframing questions

- Probable, or just possible?
- What would I tell a friend?
- Can I live with 'maybe'?
- A thought is not a fact.

Anchored Inside
COUNSELING SERVICES

Mindfulness & Acceptance Skills

1. **Notice and name:**
   "I'm having the thought that..."
2. **Label it:**
   "That's OCD talking."
3. **Urge surfing:** notice the urge rise, peak, and pass without acting.
4. **Let thoughts**
   be like clouds passing.
5. **Return to your values**
   and the present moment.
6. **Practice willingness**
   to feel uncertainty.



For Teens: Boss Back OCD



- Give OCD a name and talk back to it.
- Thoughts are just brain noise.
- Delay the ritual 5 minutes, then longer.
- Do the opposite of what OCD demands, in small steps.
- Tell a trusted adult.
- Celebrate brave moves.

WHEN IT GETS LOUD, TRY:

Breathe • 5 senses • move

You're stronger than OCD. Keep choosing YOU.

For Parents & Caregivers

Reducing Accommodation

Accommodation means joining rituals, giving repeated reassurance, or changing family routines for OCD; it brings relief but keeps OCD going.



What helps

- Stay calm and validating
- Reduce accommodation gradually with a plan made together
- Praise brave behavior
- Work with the therapist

What doesn't

- Punishing
- Arguing about the logic
- Endless reassurance

Supportive statements

"I know this feels scary, and I believe you can handle it."

"That sounds like OCD. What would your brave self do?"

"I love you too much to help OCD."

For Practitioners

Assessment

- Screen thoroughly
- Use standardized measures such as the Y-BOCS and CY-BOCS
- Assess insight, comorbidity, and family accommodation

Structure ERP

- Psychoeducation
- Collaborative hierarchy with distress ratings
- In-session exposures
- Homework
- Ritual prevention
- Address mental rituals and reassurance seeking
- Track progress



- Family involvement, especially with youth

Consider medication referral and higher levels of care for severe cases.



Quick Reference & Practice Tracker

Skills summary

Notice it • Name it • Face it • Resist the ritual • Ride the wave • Reward bravery

Day	Exposure practiced	Distress before (0–10)	Distress after	Ritual resisted?

Resources

International OCD Foundation — iocdf.org

If you are in crisis, call or text 988 Suicide & Crisis Lifeline.

This guide is educational and does not replace professional treatment.

Further reading: The OCD Workbook (3rd ed.), Hyman & Pedrick; Overcoming Your OCD, Margaret Auguste, LMFT.